

BetterResources — Executive Summary & Lean Business Plan (v1.0)

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Prepared for: Prospective Seed Investors

Company: **BetterResources** (d/b/a BetterResources App)

Contact: Heather Cochran · [email] heathersocialworker@gmail.com · [phone] []

Website / Demo:

1) One-Page Executive Summary

Mission

Make mental-health and recovery support radically more accessible by putting a verified, localized resource navigator and daily micro-coaching in everyone's pocket.

Problem

People in recovery (and their families) struggle to find *timely, trustworthy* help—meeting directories are fragmented, warmline numbers change, eligibility rules are confusing, and waitlists are long. Clinicians and case managers lose hours weekly on referrals and follow-up.

Solution — BetterResources App

A lightweight mobile app that:

- Curates **verified local resources** (treatment, crisis/warm lines, mutual-aid meetings, sliding-scale therapy, housing, legal aid, employment).
- Adds **daily micro-coaching**: triggers tracker, coping-skills checklists, and end-of-day reflections.
- Offers **guided referrals** to partners (clinics, tele-therapy, IOPs, SUD programs) with bidirectional status updates.
- Integrates external directories (e.g., meeting guides, warmline lists) via API/data partnerships.

Customer & Market

- **B2B2C:** Community clinics, IOPs, hospitals, universities, payors, EAPs who provision the app to patients/members to improve engagement and reduce care-coordination time.
- **B2C:** Individuals in recovery and caregivers seeking dependable, stigma-free help.

Business Model

- **SaaS:** \$2–\$5 PMPM for clinics/payors (tiered by features/volume).
- **Consumer:** \$7–\$10/mo optional premium (offline packs, care plans, export, reminders).
- **Affiliate/Referral Fees:** Qualified hand-offs to vetted partners (ethical, user-consented).

Go-to-Market

Start with Arizona & Southwest US: sell to community clinics/IOPs; co-marketing with crisis lines, universities, sober-living networks; partnerships with directories; founder-led sales; social proof via case studies.

Moat

Verified local data ops, consented outcomes loop with providers, and a recovery-first UX (fast, private, zero-shame). HIPAA-aware architecture and SOC 2 readiness from day one. Community brand built around a friendly “lava lamp” motif.

Funding Ask

\$2,000,000 seed to reach 12–18 months runway and hit: **200 clinic logos, 250k MAU, \$250k+ MRR** run-rate.

Use of Funds (high level)

- 35% Product & Engineering (mobile, data platform, integrations)
- 25% GTM (founder-led sales, 2 AEs, community marketing)
- 15% Data Operations (verification, QA, content partnerships)
- 10% Compliance & Security (HIPAA, SOC 2 Type I)

- 10% G&A / Ops
- 5% Contingency

Round Mechanics (suggested)

SAFE (MFN) with \$12–\$16M post-money cap **or** priced seed with standard terms. Lead check target: \$1.2M; remaining \$800k allocated to strategic angels & funds.

Team

- **Heather Cochran**, Founder/CEO — Licensed social worker; 10+ years front-line crisis and recovery support; product lead for BetterPsych/BetterRecovery apps.
- **[CTO/Lead iOS] []** — Swift/SwiftUI; healthcare data integrations; security.
- **[Head of Partnerships] []** — community health, universities, payors.
Advisors: Clinical, security, and GTM advisors to be announced (confirming).
- Project Managers

Traction (insert current metrics)

- Waitlist: []
- Pilot LOIs/MOUs: [] clinics/universities]
- Active beta users: []
- Partner directories integrated: []
- NPS/CES: []

2) Product Overview

Core Modules

1. **Resource Navigator:** geofenced, filterable, verified listings; eligibility & hours; saved favorites; one-tap call/text.
2. **Daily Tools:** triggers tracker; coping-skills checklist; gentle reminders; mood & craving logs; end-of-day reflection.
3. **Care-Team Connect (B2B):** shareable care plan, consented status updates on referrals, secure messaging to a clinic inbox.
4. **Insights:** anonymized heatmaps for partners to spot gaps (ethically, opt-in only).

Differentiators

- “Minutes-to-help” focus (speed to human support).
- Verified community data layer; freshness SLAs; change alerts.
- Recovery-first UX language; low-cog-load design; offline packs.
- Privacy: local-first storage for sensitive journaling; PHI isolation; transparent consent.

Roadmap (next 6–18 months)

- **MVP (90 days):** AZ coverage, meeting guide + warmline integrations, triggers/copings, clinic pilot v1.
- **V2 (6–9 mo):** multi-state coverage, referral workflows, SOC 2 Type I, basic analytics, web portal.
- **Scale (12–18 mo):** national coverage, Part 2 considerations, EHR/FHIR integrations, language packs (ES), payor pilots